



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : امينه ابوالعلا يوسف يوسف

العنوان : ارض الحداد

الحى / المدينة : امبابه

المحافظة : الجيزه

تاريخ تسجيل الإستماره : 29-09-2026

رقم البطاقه : 26906160101803 رقم التليفون : 01156617170

العمر (تاريخ الميلاد) : 16-06-1969 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : _____
التوقيع: (_____)

المقرر بما فيه : _____
التوقيع: (_____)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة

قسم العلاج :
الأورام

[11129]

التعليق على التشخيص : _____

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

الوظيفة : _____
الوظيفة : _____
الوظيفة : _____

التوقيع : _____
التوقيع : _____
التوقيع : _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. _____ /د. _____ /د. _____ /د. _____

قرار اللجنة

التشخيص النهائي : _____
خطة العلاج : _____
المستشفى : _____
جهة العلاج : _____
ملاحظات : _____

قسم : _____
نفقات : _____

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: امينه ابوالعلا يوسف يوسف
National ID: 26906160101803



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الطول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تخليق الشخصية

أمنته
أبو العلا يوسف يوسف
٣٨٨ ش الحكيم أول قطرة - أرض الخداد
أمنية - الجيزة

٢٠١٩/٠٦/١٦ ٢٠١٩/٠٦/١٦ ٢٠١٨/٠٣

JG3002226

٢٠٢٢/٠١ ٢٠١٩/٠٦/١٦ ٢٠١٨/٠٣

ربة منزل

أنثى
معلمة
محمّد فؤاد عبد ربّ الشّمس
البطاقة سارية حتى ٢٠٢٤/٠١/٢٨

MDT FORM

17/9/2025 G

Name	اميرة ابو الفتح	MRN	50 / 1971
Age	53	ID	110027995
Menopausal status	Post	Phone	110771414
NRS SCORE		Marital status	Married
BMI		Family history	+ve (mother)
CO morbidity		REASON OF MDT	Post Op

579.717.1.11.2

LT MRM (upfront) Clinical Data

BX: Bilocal $\leftarrow$ ID + micropapillary features

ER: 8/8 PR: 6/8 HER 2: 1+ KI67: 5%

Sonomammography:
two masses 3cm & 2cm

Mets Workup:

DCIS 10%
+ve angio invasion
Free NAC
free Deep margins
TNM
ALND: 8/13
no extranodal extension

Recommendations:

Stella
5-50/9/17

for ady AC x 4, Taxol 12w

MDT OUTCOME & PLAN

for PORT
Ady hormonal AI
& Zeta/Bu

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

atient Name : الأستاذ / أمينة ابوالعلا يوسف يوسف
atient No. : 26906160101803
ath No. : 2674/25
ex : Female
ge : 56 Y
eferreing Doc :

Request No. : 13005812
Request Date : 31/08/2025
Verification Date : 11/09/2025
Financial Category : نفقة الدولة - جوستافاروسي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Up-Front.
ature Specimen: Left modified radical mastectomy.

Gross Examination:

Left modified radical mastectomy specimen measured 25x20x4 cm, with covering skin measured 17x15 cm. Nipple & Areola looked unremarkable grossly.
Serialling showed two masses measured 3x2x1.5 cm & 2x2x1.5 cm, located 7 cm apart & were 1 cm away from the least deep margin.
Axillary fat measured 8x6 cm, entangling 13 lymph nodes, the largest was 2x1.5x1 cm.

2 Blocks:

3- 1st Mass., 4- Area Inbetween., 5 & 6- 2nd mass., 7- Nipple & areola., 8- Deep margin., 12- ALNs.

Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

Histological Type: Bifocal mixed invasive duct carcinoma with micropapillary features & invasive lobular carcinoma (Predominantly ductal component & minor lobular element).
Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
10% Intermediate grade ductal carcinoma in situ (Solid & cribriform patterns).

Angioinvasion: Present.

Perineural invasion: Absent.

Microcalcification: Present.

Nipple & areola: Free.

Deep margin: Free.

Axillary lymph nodes: Positive metastatic deposits in three out of thirteen lymph nodes (3/13), with no extra-nodal extension.

Verified by :

Dr.Duaa Mohamed

Inting Date : 14/09/2025 11:11:32

Page 1 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - MIsr El Qadema - Cairo, Egypt

Patient Name : الأمستاه / امينه ابو العلا يوسف يوسف
Patient No. : 26906160101803
Path No. : 2674/25
Sex : Female
Age : 56 Y
Referring Doc :

Request No. : 13005812
Request Date : 31/08/2025
Verification Date : 11/09/2025
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Diagnosis:

Left breast cancer, Left modified radical mastectomy:
Bifocal mixed invasive duct carcinoma with micropapillary features & invasive lobular carcinoma, Grade 2 (Predominant ductal component & minor lobular element).
10% Intermediate grade ductal carcinoma in situ (Solid & cribriform patterns).
Nipple & areola: Free.
Deep margin: Free.
Axillary lymph nodes: Positive metastatic deposits in three out of thirteen lymph nodes (3/13), with no extra-nodal extension.
Stage: pT2(m), pN1a.

Signature
Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

Printing Date : 14/09/2025 11:11:32

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Ameena Aboul Ela Youssef Youssef	Date : 28/06/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 56 Year(s)
Medical ID: 26906160101803 10080	Pat. ID: 100243

MATERIAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

- Breast density: Scattered densities of glandular tissue ...Matching ACR 8.
- Two irregular shaped masses are seen in the left upper outer quadrant and in the left lower inner quadrant with focal asymmetry is seen in the left retroareolar region .
- No detected suspicious mass density, architectural distortion or pathological micro-calcification in the right breast.
- Normal skin and nipple on the right side ..

Complementary breast ultrasound:

- Homogenous (predominantly glandular) echo-texture of both breasts.
- Two irregular shaped masses are seen in the left breast at 12 o'clock position zone 2 measuring (23 x 15 mm) and other mass at 8 o'clock ,zone 3 measuring (23 x 15 mm) .
- An area of parenchymal heterogeneity is seen in the left breast at 12-1 o'clock position ,zone 1.
- No detected suspicious solid or cystic masses in the right breast.
- Two at least suspicious looking left axillary LNs, the largest measures (15 x 9 mm) .

CONCLUSION:

- Left breast : Highly suspicious looking left Multicentric process.... BI-RADS 5 category...
- Right breast :normal BIRADS 1' category

Much Obliged

DR.MONA ZAIKY.MD

Professor-Breast imaging-Mansoura university

جمهورية قطر
وزارة الداخلية
مطابق تعليمات النسخة

أمينه
أبو العلا يوسف يوسف
١٢ من الحكم أول قطر مزارع الحداد
أمينه - الجيزة

٢٦٩.٠٦١٦.٠١.١٨.٠٣

JG3002226



MDT FORM 17/9/2025 G

Name	امينة ابو الهول	MRN	50/1971
Age	57	ID	11077995
Menopausal status	post	Phone	110771717
NRS SCORE		Marital status	Married
BMI		Family history	+ve (mother)
CO morbidity		REASON OF MDT	Post op

LT MRM (upfront)

Clinical Data

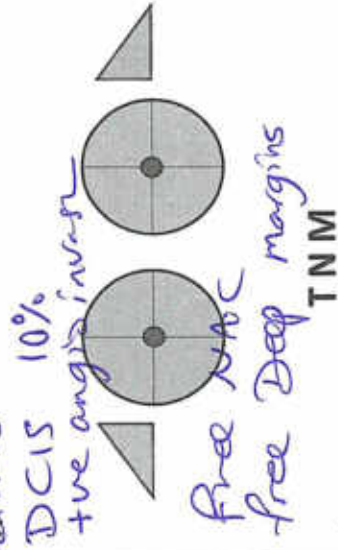
BX: Bilocal $\leq$ ID + micropapillary features

ER: 8/8 PR: 6/8 HER 2: 14 Ki67: 5%

Sonomammography:

two masses 3cm & 2cm

Mets Workup:



ALND: 3/13
no extranodal extension

Recommendations:

Locally AC x3, Taxol 12w

MDT OUTCOME & PLAN for PORT

Agg haemo and AJ & Zaka for

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	: الأستاذ / امينه ابوالعلا يوسف يوسف	Request No.	: 13005812
Patient No.	: 26906160101803	Request Date	: 31/08/2025
Path No.	: 2674/25	Verification Date	: 11/09/2025
Sex	: Female	Financial Category	: نفقة الدولة - جوستاف روسي
Age	: 56 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Up-Front.

Nature Specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 25x20x4 cm, with covering skin measured 17x15 cm. Nipple & Areola looked unremarkable grossly.
- * Serialling showed two masses measured 3x2x1.5 cm & 2x2x1.5 cm, located 7 cm apart & were 1 cm away from the least deep margin.
- * Axillary fat measured 8x6 cm, entangling 13 lymph nodes, the largest was 2x1.5x1 cm.

12 Blocks:

- 1:3- 1st Mass., 4- Area inbetween., 5 & 6- 2nd mass., 7- Nipple & areola., 8- Deep margin., 9:12- ALNs.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

- * Histological Type: Bifocal mixed invasive duct carcinoma with micropapillary features & invasive lobular carcinoma (Predominantly ductal component & minor lobular element).
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * 10% Intermediate grade ductal carcinoma in situ (Solid & cribriform patterns).
- * Angioinvasion: Present.
- * Perineural invasion: Absent.
- * Microcalcification: Present.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Positive metastatic deposits in three out of thirteen lymph nodes (3/13), with no extra-nodal extension.

Verified by :

Dr.Duaa Mohamed

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Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / امينه ابوالعلا يوسف يوسف :
Patient No. : 26906160101803
Path No. : 2674/25
Sex : Female
Age : 56 Y
Referreing Doc :

Request No. : 13005812
Request Date : 31/08/2025
Verification Date : 11/09/2025
Financial Category : فقة الدولة - جوستاف روسي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Diagnosis:

Left breast cancer, Left modified radical mastectomy:
Bifocal mixed invasive duct carcinoma with micropapillary features & invasive lobular carcinoma, Grade 2 (Predominant ductal component & minor lobular element).
- 10% Intermediate grade ductal carcinoma in situ (Solid & cribriform patterns).
- Nipple & areola: Free.
- Deep margin: Free.
- Axillary lymph nodes: Positive metastatic deposits in three out of thirteen lymph nodes (3/13), with no extra-nodal extension.
- Stage: pT2(m), pN1a.

Signature

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

Printing Date : 14/09/2025 11:11:32

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / أمينة أبو العلا يوسف يوسف : Request No. : 13004002
 Patient No. : 26906160101803 : Request Date : 02/07/2025
 Path No. : 2192/25 : Verification Date : 07/07/2025
 Sex : Female : Financial Category : نفقة الدولة - أورام دار السلام
 Age : 56 Y : Department : Out Patient
 Referreing Doc : : Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data

Left breast mass

Nature of specimen

Tru cut biopsy, collected on 1/7/2025

Gross examination

Multiple grayish white tissue cores up to 1.5 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for immune-staining using Bench

Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), &

Ki67 clone (20-6).

Microscopic examination

* Histological type: Mixed invasive duct & lobular carcinoma
 * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade 2

* DCIS: Absent

* Angioinvasion: Absent

* Perineural invasion: Absent

* Additional pathological findings : Positive microcalcification.

* IPT (Positive control showed appropriate reactivity) :

* ER: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)

* PR: Strong nuclear reaction in 20 % tumor cells (Allred score 6/8)

* HER2: Faint incomplete membrane reaction in <10 % of tumor cells (score 1+)

* Ki 67: 5 % nuclear reaction of tumor cells

Diagnosis

Verified by :

Dr. Heba Hasan

صالح
طاهر
سليم

Printing Date : 6907/2025 09:09:30

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name : Mrs Amina Abuelela
Date: 11 December 2025

Duplex Examination of the venous systems of The lower limbs

Deep Venous System: -

- > Patent both CFVs , SFVs , POP v all seen compressible who no DVT detected on both sided
- > Patent deep calf veins with no DVT detected

Superficial Venous System: -

- > Patent long & short saphenous veins on both sides with no evidence of superficial thrombophlebitis.
- > Mild diffuse edema of lower limbs more evident at left thigh with patchy skin inflammatory lesions ?? allergic for clinical correlation with follow up .

OPINION: -

NO DVT nor superficial thrombophlebitis.

Best regards

Dr. Abubakr Hejazi MSc FEBR

Patient Name: Ameena Aboul Ela Youssef Youssef	Date : 26/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID:	Pat. ID: 100243

POST CONTRAST C.T. SCAN OF THE CHEST:-

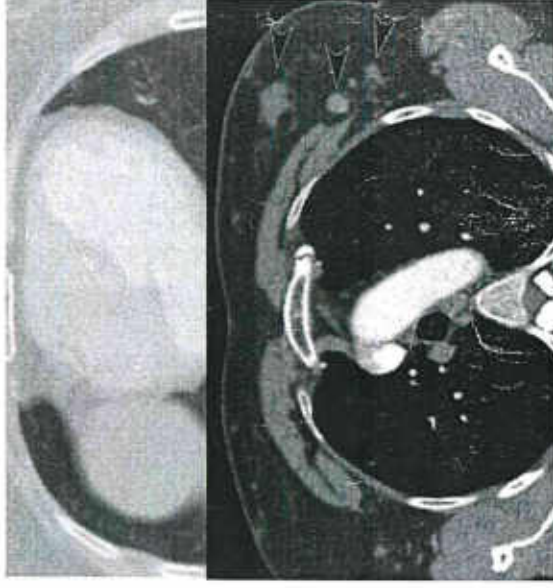
TECHNIQUE: Axial contiguous thin cuts were taken through the chest after I.V. contrast injection.

FINDINGS:

- *Mild bilateral diffuse mosaic appearance denoting small air way disease.*
- *Middle lobar and lingular thick atelectatic bands*
- The rest of both lungs show normal appearance of its parenchyma and pulmonary vasculature with no evidence of abnormal masses, nodules or consolidative patches
- Normal C.T. appearance of the mediastinum vessels.
- *Few small right axillary and aortopulmonary lymph nodes.*
- Patent tracheo bronchial tree.
- No pleural or pericardial effusion.
- Average size of the heart is noted.
- The included lower neck cuts are unremarkable
- *An ill-defined left upper inner breast mass is seen measuring about 13 x 24 mm with multiple globular left axillary lymph nodes largest measuring about 14 x 8 mm.*

CONCLUSION :-

- Mild bilateral diffuse mosaic appearance denoting small air way disease.
- Middle lobar and lingular thick atelectatic bands
- An ill-defined left breast mass with multiple globular left axillary lymph nodes ,for correlation with sonomammography.



Patient Name: Aameena Aboul Ela Youssef Youssef	Date : 26/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID:	Pat. ID: 100243

POST CONTRAST C.T. SCAN OF THE ABDOMEN AND PELVIS:-

TECHNIQUE: Axial contiguous thin cuts were taken through the abdomen and pelvis after I.V. contrast injection.

FINDINGS:

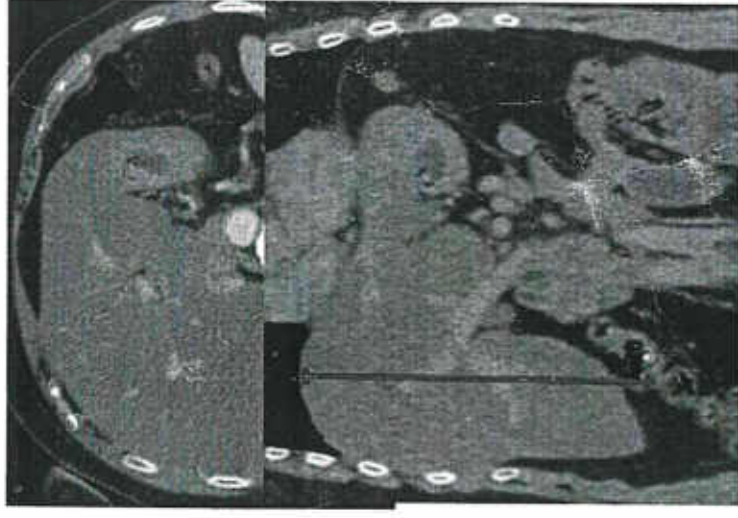
- Liver is **enlarged with diffuse decreased hepatic attenuation** yet with no focal lesions .
- No extra or intra-hepatic biliary dilatation .
- Both kidneys are functioning and normal in dimensions.
- The spleen and pancreas show normal C.T. appearance .
- Normal size and C.T. density of the adrenal glands .
- No ascites .
- The urinary bladder shows normal size and wall thickness with no calculi inside.
- The uterus is of normal C.T. appearance .
- Normal ischeo-rectal fossae on both sides .
- No sizable abdominal or pelvic lymph glands.
- **No suspicious bony lesions.**

IMPRESSION:

Fatty hepatomegaly for correlation with lipid and hepatic profile.

Much Obliged,

Mona Ali, MD



Patient Name: Ameena Aboul Ela Youssef Youssef	Date : 28/06/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 56 Year(s)
Medical ID: 26906160101803 10080	Pat. ID: 100243

BIATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

- Breast density: Scattered densities of glandular tissue ...Matching ACR B.
- Two irregular shaped masses are seen in the left upper outer quadrant and in the left lower inner quadrant with focal asymmetry is seen in the left retroareolar region .
- No detected suspicious mass density, architectural distortion or pathological micro-calcification in the right breast.
- Normal skin and nipple on the right side ..

BY COMPLEMENTARY breast ultrasound:

- Homogenous (predominantly glandular) echo-texture of both breasts.
- Two irregular shaped masses are seen in the left breast at 12 o'clock position zone 2 ,measuring (23 x 15 mm) and other mass at 8 o'clock ,zone 3 ,measuring (23 x 15 mm).
- An area of parenchymal heterogeneity is seen in the left breast at 12-1 o'clock position ,zone 1.
- No detected suspicious solid or cystic masses in the right breast.
- Two at least suspicious looking left axillary LNs, the largest measures (15 x 9 mm).

CONCLUSION:

- Left breast : Highly suspicious looking left Multicentric process.... BI-RADS 5 category...
- Right breast :normal BIRADS 1` category

Much Obliged

DR.MONA ZAKY.MD

Professor-Breast imaging-Mansoura university

Name : Mrs Amina Abuelela

Date: 11 December 2025

Duplex Examination of the venous systems of The lower limbs

Deep Venous System: -

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- Patent deep calf veins with no DVT detected

Superficial Venous System: -

- Patent long & short saphenous veins on both sides with no evidence of superficial thrombophlebitis.
- Mild diffuse edema of lower limbs more evident at left thigh with patchy skin inflammatory lesions ?? allergic for clinical correlation with follow up .

OPINION: -

NO DVT nor superficial thrombophlebitis.

Best regards

Dr. Abubakr Hejazi MSc FEBR

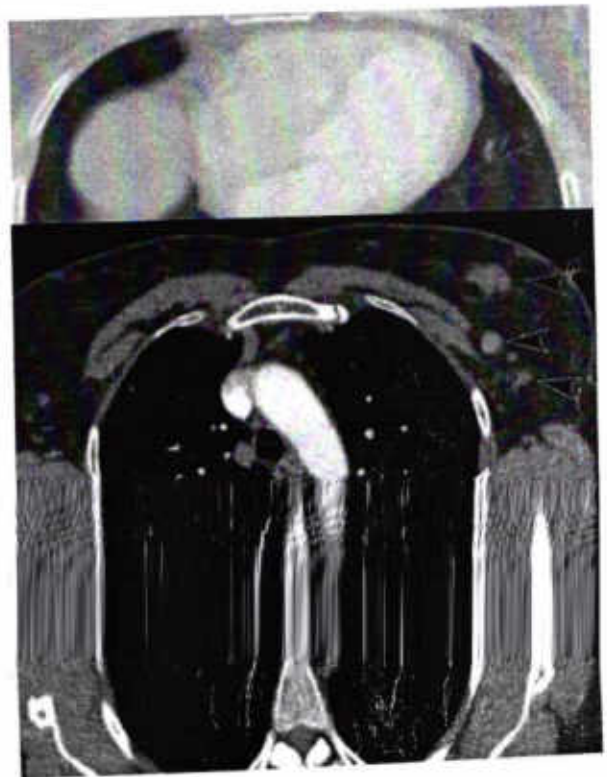
Patient Name: Ameena Aboul Ela Youssef Youssef	Date : 26/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID:	Pat. ID: 100243

POST CONTRAST C.T. SCAN OF THE CHEST:-

TECHNIQUE: Axial contiguous thin cuts were taken through the chest after I.V. contrast injection.

FINDINGS:

- *Mild bilateral diffuse mosaic appearance denoting small air way disease.*
- *Middle lobar and lingular thick atelectatic bands*
- The rest of both lungs show normal appearance of its parenchyma and pulmonary vasculature with no evidence of abnormal masses, nodules or consolidative patches
- Normal C.T. appearance of the mediastinum vessels.
- *Few small right axillary and aortopulmonary lymph nodes.*
- *Patent tracheo bronchial tree.*



- No pleural or pericardial effusion.
- Average size of the heart is noted.
- The included lower neck cuts are unremarkable
- *An ill-defined left upper inner breast mass is seen measuring about 13 x 24 mm with multiple globular left axillary lymph nodes largest measuring about 14 x 8 mm.*

CONCLUSION :-

- Mild bilateral diffuse mosaic appearance denoting small air way disease.
- Middle lobar and lingular thick atelectatic bands
- An ill-defined left breast mass with multiple globular left axillary lymph nodes ,for correlation with sonomammography.

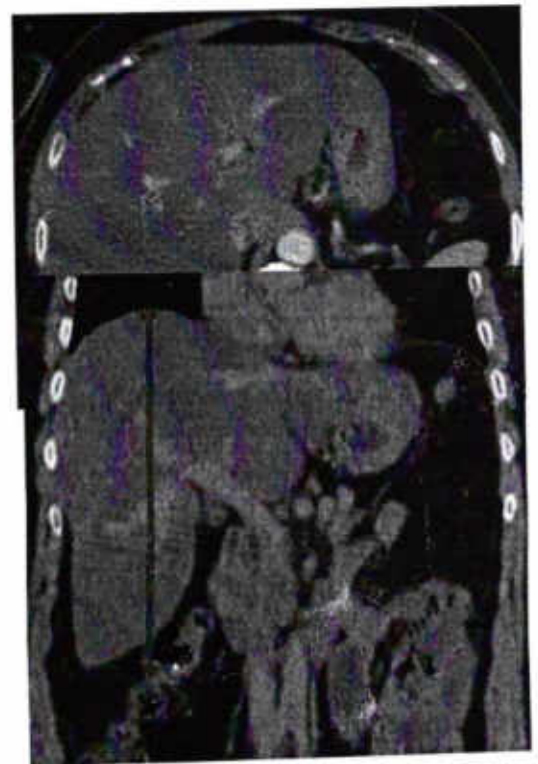
Patient Name: Ameenah Aboul Ela Youssef Youssef	Date : 26/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID:	Pat. ID: 100243

POST CONTRAST C.T. SCAN OF THE ABDOMEN AND PELVIS:-

TECHNIQUE: Axial contiguous thin cuts were taken through the abdomen and pelvis after I.V. contrast injection.

FINDINGS:

- Liver is *enlarged with diffuse decreased hepatic attenuation yet* with no focal lesions .
- No extra or intra-hepatic biliary dilatation .
- Both kidneys are functioning and normal in dimensions.
- The spleen and pancreas show normal C.T. appearance .
- Normal size and C.T. density of the adrenal glands .
- No ascites .
- The urinary bladder shows normal size and wall thickness with no calculi inside.
- The uterus is of normal C.T. appearance .
- Normal ischio-rectal fossae on both sides .
- No sizable abdominal or pelvic lymph glands.
- *No suspicious bony lesions.*



IMPRESSION:

Fatty hepatomegaly for correlation with lipid and hepatic profile.

Much Obligated,

Mona Ali,MD

Patient Name: Ameena Aboul Ela Youssef Youssef	Date : 28/06/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 56 Year(s)
Medical ID: 26906160101803 10080	Pat. ID: 100243

BIATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

- **Breast density:** Scattered densities of glandular tissue ...Matching ACR B.
- **Two irregular shaped masses are seen in the left upper outer quadrant and in the left lower inner quadrant with focal asymmetry is seen in the left retroareolar region .**
- No detected suspicious mass density, architectural distortion or pathological micro-calcification in the **right breast**.
- Normal skin and nipple on the right side ..

by complementary breast ultrasound:

- Homogenous (predominantly glandular) echo-texture of both breasts.
- **Two irregular shaped masses are seen in the left breast at 12 o'clock position ,zone 2 ,measuring (23 x 15 mm) and other mass at 8 o'clock ,zone 3 ,measuring (23 x 15 mm).**
- **An area of parenchymal heterogeneity is seen in the left breast at 12-1 o'clock position ,zone 1.**
- **No detected suspicious solid or cystic masses in the right breast.**
- **Two at least suspicious looking left axillary LNs, the largest measures (15 x 9 mm).**

CONCLUSION:

- **Left breast : Highly suspicious looking left Multicentric process.... BI-RADS 5 category...**
- **Right breast :normal BIRADS 1' category**

Much Obliged

DR.MONA ZAKY.MD

Professor-Breast imaging-Mansoura university



Patient Name : الأستاذة / امينه ابوالعلا يوسف يوسف
Patient No. : 26906160101803
Path No. : 2674/25
Sex : Female
Age : 56 Y
Referring Doc :

Request No. : 13005812
Request Date : 31/08/2025
Verification Date : 11/09/2025
Financial Category : نفقة الدولة - جوستاف روسي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Up-Front.
Nature Specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 25x20x4 cm, with covering skin measured 17x15 cm. Nipple & Areola looked unremarkable grossly.
- * Serialing showed two masses measured 3x2x1.5 cm & 2x2x1.5 cm, located 7 cm apart & were 1 cm away from the least deep margin.
- * Axillary fat measured 8x6 cm, entangling 13 lymph nodes, the largest was 2x1.5x1 cm.

12 Blocks:

- 1:3- 1st Mass., 4- Area inbetween., 5 & 6- 2nd mass., 7- Nipple & areola., 8- Deep margin.,
9:12- ALNs.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

- * Histological Type: Bifocal mixed invasive duct carcinoma with micropapillary features & invasive lobular carcinoma (Predominantly ductal component & minor lobular element).
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * 10% Intermediate grade ductal carcinoma in situ (Solid & cribriform patterns).
- * Angioinvasion: Present.
- * Perineural invasion: Absent.
- * Microcalcification: Present.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Positive metastatic deposits in three out of thirteen lymph nodes (3/13), with no extra-nodal extension.

Verified by :
Dr.Duaa Mohamed





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Registered by : Mrs.Mayada Hegazy

Pathology Report

Diagnosis:

Left breast cancer, Left modified radical mastectomy:

Bifocal mixed invasive duct carcinoma with micropapillary features & invasive lobular carcinoma, Grade 2 (Predominant ductal component & minor lobular element).

- 10% Intermediate grade ductal carcinoma in situ (Solid & cribriform patterns).

- Nipple & areola: Free.

- Deep margin: Free.

- Axillary lymph nodes: Positive metastatic deposits in three out of thirteen lymph nodes (3/13), with no extra-nodal extension.

- Stage: pT2(m), pN1a.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed



ECHO REPORT

Name: امينه ابو العلاء

Date: 30 / 9 / 2025

EF	64.1	55-75%	PWT	0.7	0.6-1.1cm
LVEDD	4.1	3.5-5.8cm	AO.R	2.8	2-3.7cm
LVESD		2-4cm	LA	3.6	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions normal
 LV wall thickness normal
 LV diastolic function DD grade II
 SWMA SWTA
 LA and Ao root dimensions normal
 Mitral Valve Mild MR
 Aortic valve Mild AR
 Tricuspid valve Mild TR RUSP 2mmHg
 Pulmonary valve normal
 Right side and systolic function normal
 Intracardiac masses and effusion normal

Dr /

ECHO
DOCTYRE ECHO

جمهورية مصر العربية
بطاقة تحقيق الشخصية



امينة
ابو العلا يوسف يوسف
١٣ ش الحكيم اول قطر مارض الحدا
امبليه - الجيزه

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J63002226



Patient Name : الأستاذة / امينه ابوالعلا يوسف يوسف	Request No. : 13004002
Patient No. : 26906160101803	Request Date : 02/07/2025
Path No. : 2192/25	Verification Date : 07/07/2025
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 56 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs Mayada Hegazy

Pathology Report

Clinical Data

Left breast mass

Nature of specimen

Tru cut biopsy, collected on 1/7/2025

Gross examination

Multiple grayish white tissue cores up to 1.5 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination

- * Histological type: Mixed invasive duct & lobular carcinoma
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade 2
- * DCIS: Absent
- * Angiolysis: Absent
- * Perineural invasion: Absent
- * Additional pathological findings : Positive microcalcification
- * IPT (Positive control showed appropriate reactivity) :
- * ER: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
- * PR: Strong nuclear reaction in 20 % tumor cells (Allred score 6/8)
- * HER2: Faint incomplete membrane reaction in <10 % of tumor cells (score 1+)
- * Ki67: 5 % nuclear reaction of tumor cells

Diagnosis

Verified by :

Dr. Hiba Hasan

منقح ملف
د. هبة حسن
١٩/٧



Patient Name : الأستاذ / أمينة أبو العلا يوسف يوسف
Patient No. : 26906160101803
Path No. : 2192/25
Sex : Female
Age : 56 Y
Referring Doc :

Request No. : 13004002
Request Date : 02/07/2025
Verification Date : 07/07/2025
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Left breast mass, Tru cut biopsy:

Mixed Invasive duct & lobular carcinoma, Grade 2

IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 6/8).
- * HER2: Negative (score 1+).
- * Ki67: Positive in 5% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoenim
Dr. Heba Hassan Adly

Verified by :
Dr. Heba Hasan

Printing Date : 09/07/2025 09:09:30

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



ECHO REPORT

Name: امينة ابوالعلا

Date: 25 / 8 / 2025

EF	64%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.5	2-3.7cm
LVESD		2-4cm	LA	3.6	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions ——— normal

LV wall thickness ——— normal

LV diastolic function DD Grade I

SWMA ——— no SWMA, However IHD cannot be excluded

LA and Ao root dimensions ——— normal

Mitral Valve Mild MR

Aortic valve Mild AR

Tricuspid valve Mild TR

Pulmonary valve ——— normal

Right side and systolic function ——— normal

Intracardiac masses and effusion ——— normal

Dr. Hisho